جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : عطيات محمد السيد حسن
المحافظة : القاهره **الحي / المدينة :** الدرب الاحمر **العنوان :** =
رقم البطاقة : 27201160100742 **رقم التليفون :** 01007633571 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 16-01-1972 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعه مقطعيه علي الصدر بالصبغه [10851] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عطيات محمد السيد حسن
National ID: 27201160100742

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced chest CT

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢ فرد زندعيه



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 6/10/2025 G

Name	[illegible]	MRN	٣٨٨٠/C١٩
Age	post p	ID	٢٨٤٠١١٦٠١٠٠٧٤٤
Menopausal status	50 y.	Phone	01102428858
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	HTN	REASON OF MDT	

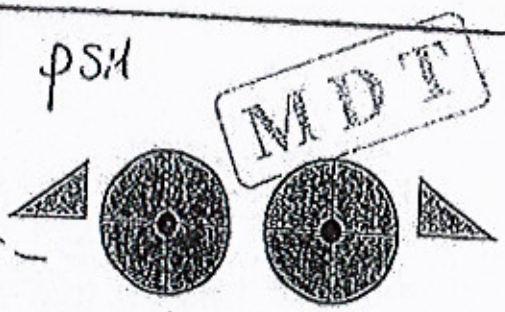
TAH, BSO ✓

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki67:
tr 8/8	+ 7/8	3+	

R psm L



Sonomammography: premen. left BIRADS 4a.

Mets Workup: LBC Sn 11/2019
T1, N0 0/12, M0
Rec 4 AC 11/12 Taxol.
pCR 17 cycles therapeutic adm.

TNM

Recommendations: Sx TAH 3/2022 → 3/2022
SHx to female. a/r Ted Lee TAH, BSO for f 3/2022
? MDT for further plan with Dr [illegible]

MDT OUTCOME & PLAN

20/10/10

Complete Sgx female Sx stage G F
keep under follow up (+ Brachy 4 cycles).

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[signature] 20/10/10		Nermin Ramadan			

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name: Atteyat Mohamed Al Sayed Hassan	Date : 06/12/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 2885/2019	Pat. ID: 11144

BILATERAL BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left breast conservative surgery.

- Homogeneous glandular back ground parenchyma.
- **Clear left operative bed with 9 o'clock ,zone 3 small seroma formation measuring (20 x 6 mm)(TR X AP diameters). .No suspicious masses .**
- ***No detected suspicious solid or cystic lesions on both sides.***
- ***No dilated retroareolar ducts.***
- Normal skin and nipple.
- No detected significant axillary lymph nodes

CONCLUSION:

- ***Left breast : Post conservative changes ....BIRADS 2 category.***
- ***Right breast : Normal study.... BIRADS 1 category.***

Much Obliged

DR. MONA ZAKY.MD

Professor of diagnostic radiology-Breast imaging unit -Mansoura University

Signature Dr. Mona Zaki

06/12/2025 2:17 PM

Mammogram
[[DOCTYPE: MAMMOGRAM]]

Name : Aghtiyat Muhammad Al-Sayed
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 21-08-2025 11:10
Medical No. : 182677

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- A 53-year-old lady, with history of left breast cancer.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Patient Name: Atteyat Mohamed Al Sayed Hassan	Date : 29/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 2885/2019	Pat. ID: 11144

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:
Left CBS

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:
- Fairly dense parenchyma, (ACR B).

LEFT BREAST:
- Focal area of parenchymal distortion is seen in the LIQ associated with subtle skin and trabecular thickening, owing to the prior surgical interference.
- Complementary ultrasound showed an underlying well defined thick walled cystic collection, located at 8-9 o'clock, measuring about 2.5x1.5cm.
- Few benign calcifications are noted.
- No definite pathological axillary node.

RIGHT BREAST:
- No dominant mass, spiculation or suspicious microcalcifications.
- Few benign calcifications are noted.
- Normal skin thickness and nipple-areola complex.
- Non-specific axillary nodes.

OPINION:
- Right breast post operative sequel, (BIRADS 2).
- Left benign calcifications, (BIRADS 2).

Yours truly,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

[[DOCTYPE:ULTRASOUND]]
Ultrasound

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name Mrs attyat Muhamed	Date : 20 /11 /2025
Exam : ultrasound ABDOMEN& PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Normal in size (14.5 cm at Rt MCL) displaying homogenous parenchymal echo pattern with smooth surface.*
- *No definite focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- *SURGICALLY REMOVED .*
- *CBD is not dilated.*

Spleen: -

- *Normal size 11cm , homogenous texture with no focal lesions, splenic veins Not dilated*

- **Kidneys: -**
  - *Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.*
  - *No renal stones or back pressure changes*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites.

Urinary Bladder: -

- *Average capacity with no stones or diverticular ~~one~~ pouching*

Uterus and adenexa*:*

- *TAH*
- *No PELVIC MASSES*

Opinion:

- **Moderate exaggerated colonic gases.**

Dr Abubakr Hejazi MSc FEBR

[[DOCTYPE:ULTRASOUND]]
ultrasound

Dar El-Salam Caner Center
Radiology Department

مركز أورام دار السلام
قسم الأشعة

Name: atiat mohamed
Date : 29 January 2020

Duplex Examination of the venous system of the right upper limb revealed: -

Venous system:

- *The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.*
- *No evidence of D.V.T*
- *Both cephalic and basilic veins are patent, no evidence of superficial thrombophilibities*
- *Mild interstitial edema of the dorsum of the hand*

INION: -

Normal venous study

Best Regards,
Dr; Nermeen Ramadan MD

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name Mrs attyat Muhamed
Exam : ultrasound ABDOMEN& PELVIS

Date : 20 /11 /2025

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

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No ascites.

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adenexa:

- *TAH*
- *No PELVIC MASSES*

Opinion:

- **Moderate exaggerated colonic gases.**

Dr Abubakr Hejazi MSc FEBR

Ultrasound
[[DOCTYPE:ULTRASOUND]]





ERM036746208

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
/ مركز اورام دار السلام

Echo report

Name: عطيات محمد السيد
Date : 5/1/2020

EF	71%	55-75%	PWT	0.6-1.1cm
LVEDD		3.5-5.8cm	AO.R	2-3.7cm
LVESD		2-4cm	LA	2-4cm
FS		25-45%	RV	0 7-2 3cm(supine
SW		0 6-1 1cm	AO CUSP	1.5-2.6cm

Systolic function with normal internal dimension EF 71%

Normal diastolic function.

Normal LA dimensions

Normal Ao root dimensions.

Normal Mitral Valve morphology

Normal Aortic valve morphology

Normal Tricuspid valve morphology and leaflets

Normal Pulmonary Valve.

No intrcardiac masses or pericardial effusion

Conclusion: Normal Echo

DR. Mohammed A. Wahab

Echo
[[DOCTYPE:ECHO]]

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مركز اورام دار السلام

Echo report

Name: محمد السيد عطيه م.
Date: 7 /IV 2020

EF	63	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.4	3.5-5.8cm	AO.R	3.1	2-3.7cm
LVESD	2.9	2-4cm	LA	3.8	2-4cm
FS	32	25-45%	RV	—	0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP	—	1.5-2.6cm

LV dimensions Normal.
LV wall thickness Normal
LV diastolic function GIDD
SWMA Normal.
LA and Ao root dimensions normal.
Mitral Valve Thickened & Trivial MR.
Aortic valve Normal
Tricuspid valve Normal & mild TR, normal RVSP.
Pulmonary valve Normal
Right side and systolic function Normal
Intracardiac masses and effusion Nil.

Mohammed Anwar, Msc cardiology.

[[DOCTYPE:ECHO]]

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m.7000

اسم المريض	: عطيات محمد السيد		
السن	:		
تاريخ اخذ العينة	:		
طالب التحليل	: مركز اورام دار السلام	النوع	: Female
عنوان	: مركز اورام دار السلام	العنوان	: القاهرة
		تاريخ استلام العينة	: 17/12/2019 13:10
		تاريخ التقرير	: 24/12/2019 12:32

Primary Sample :

Paraffin Block

Test :

HER2 by SISH

Comment :

Technique :

sample received was stained for HER2 gene and chromosome 17 using INFORM HER2 Dual Probe Cocktail technique in benchmark GX (VENTANA).

Result :

the ratio of HER2 signal to chromosome 17 in the nuclei of tumor cells is 2.5 (more than 2) indicating amplification of HER2 gene

Referal lab :

Comment :

Conclusion :

#refered block : HER2 GENE AMPLIFIED. RATIO 2.5 (LESS THAN 2)

Pathologist: د/تريزا ماهر

Lab Director: د/ تريزا ماهر

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website:http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor [illegible]

- مدة العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry
[[DOCTYPE:IHC]]

DAR EL SALAM CANCER CENTER
RADIOLOGY DEPARTMENT
ULTRASOUND UNIT

مـــركـــز اورام دار الســـــلام
قســـم الاشـعـة التشخيصية
وحـــدة المـــوجـات الصـــوتية

Name; atiat mohamed elsayed
Date: 17 August 2022

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size homogenous parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic billiary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with multiple small stones seen inside
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size , homogenous.*

- **Kidneys: -**
  - *Both are average in size,shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adenexeal *; uterus is surgically removed*

Opinion:

Calcular GB
Hysterectomy
Colonic gaseous distention

Best regards
Dr/nexmeen Ramadan MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

معمل صبغات مناعيه

Oncology Center مركز الأورام

خطابات أورام

Patient Name : عطيات محمد السيد **sex :** female **Age:** 47

Hosp.Word : CT **Patient status :** outpatient

Path.No. : **IHC . No :** 12071

Referring Doctor: السلام-هرمل **Date :** 18-11-2019

Provisional Diagnosis: request for Ki67.

Immunohistochemistry Report

CLINICAL/ RADIOLOGIC DATA
Breast cancer.

IMMUNOHISTOCHEMISTRY
Immunostaining effectuated on 5 µm thick paraffin sections, the heat induced epitope retrieval using microwave, the ChemMate DAKO Envision, DAKO autostainer and the following antibody:
- Ki67 : more than 20% of the neoplastic cells are positive.

CONCLUSION
Ki67, high proliferation fraction.

Enclosed :

Slides : **Blocs :**

Director Immunohistochemical Lab
Prof.Howayda Abd El All
23-11-2019

مدير معمل الصبغات الهستومناعيه
ا. د / هويدا عبدالعال

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039164 - 22039356 Fax: 22039 236

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

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Director Immunohistochemical Lab
Prof.Howayda Abd El All
23-11-2019

مدير معمل الصبغات الهستومناعيه
ا. د / هويدا عبدالعال

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039164 - 22039356 Fax : 22039 236

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m.7000

		النوع : Female	
		العنوان : القاهرة	
		تاريخ استلام العينة : 17/12/2019 13:10	
مكان التحليل	مركز اورام دار السلام	تاريخ التقرير : 24/12/2019 12:32	
	مركز اورام دار السلام		

Primary Sample :

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Test :

HER2 by SISH

Comment :

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Referal lab :

Comment :

Conclusion :

Prefered block : HER2 GENE AMPLIFIED. RATIO 2.5 (LESS THAN 2)

Pathologist: د/تريزا ماهر

Lab Director: د/ تريزا ماهر

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facebook: قسم الباثولوجى - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor

صلاحية العينة 30 يوم من تاريخ استلامها

Immunohistochemistry
[[DOCTYPE:IHC]]

أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
إداره المعمل
قسم الباثولوجى

Pathology Report

Name: عطيات محمد السيد حسن **Age: 47 years.**

Clinical data: Left breast mass.

Specimen type: Sector of mammary tissue. Lft WLE + SLN

Date: 11/11/2019

Lab. Number: 1197 **Hospital Number: 3885/2019**

Gross Description:

Two specimens were received:

A sector of mammary tissue; 7.2x6.5x3 cm with partial skin covering; 5.2x3 cm. The specimen was designated by short thread as superior margin, long thread as lateral one. Serial cuts reveled an ill-defined greyish white mass; 1.8x1.5 cm located in lower inner quadrant about; 2.5 cm deep to skin and (4, 3.5, 2, 1.5 7 1.8) cm away from superior, lateral, medial, inferior and deep) margins respectively.

b) Fibro-fatty tissue pieces collectively 12 nodules, the largest was; 1.5 cm in diameter.

Microscopic Examination:

Sections examined revealed breast tissue infiltrated by invasive malignant tumor formed of nests and trabeculae of moderately anaplastic ductal epithelial cells with desmoplastic stroma.

Minimal intraductal component (<5%).

Inconspicuous perineural invasion and lymphovascular emboli.

Free nipple, areola and resection margins.

The examined axillary lymph nodes were free of deposits (0/12).

Diagnosis:

Left breast mass, sector of mammary tissue;

- **Incisive duct carcinoma GII.**
- **Minimal intraductal component (non-comedo <5%).**
- **Inconspicuous perineural invasion and lymphovascular emboli.**
- **Free skin and soft tissue margins.**
- **Free axillary lymph nodes (0/12).**
- **Tumor stage pT1 N0.**

Prof. Dr. Lubna Omar *Prof. Dr. Essam Ayad*

Dr. Enas Mohammed Ali

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / عطيات محمد السيد حسن الملا	Request No.	: 138366
Patient No.	: 27201160100742	Request Date	: 04/10/2021
Path No.	: 2316/21	Verification Date	: 11/10/2021
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referreing Doc	:		

Pathology Report

Clinical data:
Known patient of breast cancer, recieved chemotherapy.
Endometrail hyperplasiá (by D&C).

ure of specimens:
ysterectomy

GROSS: Hystrectomy specimen measured as follow: uterrine corpus 6x5x1cm, cervix measured 3x1.5cm, each ovary measured 2.5x1.5cm and each tube 5cm long.On opening enometrial thickness was 0.2cm and myometrial thickness 2cm.
- Myometrial showed trabeculation.
- Cervix showed nabothian follicles.

MICROSCOPY
histopatholgical examination of sections prepared from the endometrium shows increased G/S ratio, some glands were cystically dilated, lined by columnar cells within spindled st[illegible]na. The cervix shows evidence of chronic nonspecific cervicitis with Nabothian cyst formation. The ovary and tube show unremarkable changes. There is no evidence of malignancy in sections examined.

DIAGNOSIS:
Endometrial hyperplasia, Hysterectomy:
- Simple endometrial hyperplasia.
- Chronic ceruicitis.
- Unremarkable ovaries and tubes.

Prof. Gamal Abdelaty
Dr. Rana Fathy

[illegible] Room No. (4) - First Floor Building No. 997 - Nile Corniche - Nile El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Atteyat Mohamed Al Sayed Hassan	Date : 15/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID: 3885/2019	Pat. ID: 11144

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast LIQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and deeply seated ill defined dense mass lesion.
- Complementary ultrasound showed left breast LIQ focal parenchyma distortion, owing to prior surgical interference with underlying rather stationary well defined thick walled cystic lesion with internal echogenic secretions and avascular septations located at 8-9 o'clock measuring about 40 x15mm, no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left axillary, infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Rather stationary left LIQ organized seroma.
- Normal study of the right breast (BIRADS 1)

Signature Dr. Nehal Hamdy

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Mammogram
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Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
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Date : 19/09/2023	Patient Name: Atteyat Mohamed Al Sayed Hassan
Age : 51 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 11144	Medical ID: 3885/2019

Name: attiat Mohamed file : 3885/2019
Date: 19 September 2023

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Mildly enlarged in size (about 17 cm at the right MCL), smooth outline, bright echopattern.*
  No intra hepatic biliary radicle dilatation.
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *removed*
- *CBD is not dilated.*

Spleen: -

- *Average size , displaying homogenous echopattern.*

Kidneys: -

Are of normal size , site , well differentiated & with no gross stones or backpressure changes , no solid or cystic lesion.
-Para-aortic and pancreatic area: no gross lesion
-No ascites .

URINARY BLADDER*: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.*

Uterus *: removed*

Opinion:

Mild bright hepatomegaly (fatty).

Signature Dr. Doaa Abdelaziz
19/09/2023 12:01 PM

Ultrasound
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